STAMUN 2025 Committees

- UNESCO
- UNHRC
- UNEP
- DISEC
- WHO

MUN Procedures

- Committee Session
    1) Roll Call
        - All countries called, delegate responds with “Present” or “Present and Voting”
    2) General Speaker’s List
        - Chosen delegates give opening speech
    3) Moderated Caucus
        - Speaking rules apply
    4) Unmoderated Caucus
        - Informal session, delegates can walk around and talk to other delegates
    5) Drafted Resolutions
        - Done with multiple delegates
    6) Voting
        - Delegates vote for best resolution

MUN Terminology

- Moderated Caucus
    - Short statements/comments from delegates on sub-issues
    - If someone wants to speak on the subject, they must raise their placard and wait for the chair to call on them
- Unmoderated Caucus
    - Free movement
    - Talk to other delegates
    - Form alliances
    - Brainstorm resolutions

- Floor
    - Opportunity to speak
- Motion
    - Suggest an action
        - Extend debate time
        - Vote on resolutions
- Second
    - Agree with another delegate's motion
- Objection
    - Disagree with another delegate's motion
- Abstain
    - If a delegate is unsure about their vote they can choose not to say yes or no definitively
        - Does not fully agree
        - Agrees enough not to vote against

MUN Points

- Question or suggestion made by a delegate to the chair
- Usually regarding their comfort
- Types of points :
    - Point of Order
        - Objection to a procedure
        - The chair has made a mistake
        - Parliamentary Procedures are not done correctly
        - Must be made
        - Must say point of order whilst raising their placard
        - May interrupt a speaker who has the floor

- Point of Inquiry
    - For clarification
- Point of Information
    - To add onto another speaker statement/resolution
    - To bring down the resolution
    - Must be formulated as a short question
    - May NOT interrupt speaker who has the floor
    - Must be directed to the chair, even though it is aimed at the speaker
    - Max 2 times
- Point of Personal Privilege
    - When a delegate feels uncomfortable
    - Room too hot / too cold
    - May interrupt speaker to request for change in atmosphere
- Point of Parliament Inquiry
    - Point of info directed at chair regarding the rules of procedure

Key Terms

Term	*Category*	*Definition*
Parity	**A - System & Policy Reform**	Achieving equality in ● Insurance coverage ● Funding ● Public importance Between mental and physical health
Integration of Care	**A - System & Policy Reform**	Incorporating mental health services into : ● Primary healthcare ● Schools ● Workplaces ● Social services
Universal Access	**A - System & Policy Reform**	Ensuring timely and affordable access to mental health services. Regardless of : ● Socioeconomic status ● Location
Decentralization	**A - System & Policy Reform**	Moving away from centralized institutions and moving towards community-based care to decrease the strain on the overall health system
Health Equity/Disparity	**A - System & Policy Reform**	The capacity of mental health systems to ● Prepare for crises ● Respond to crises ● Recover from crises
Psychosocial Support	**A - System & Policy Reform**	Framework for providing mental health and psychosocial support during emergencies and recovery
Telepsychiatry	**B - Service Delivery & Technology**	The use of technology to facilitate the deployment of remote mental health services (such as online counselling)
Stepped Care Model	**B - Service Delivery & Technology**	Model begins with least intensive, most effective treatment before opening up options to “step-up” and receive more intensive care (if appropriate)
Crisis Services	**B - Service Delivery & Technology**	A range of services (from prevention and early intervention to specialized treatment and crisis support (e.g. hotlines, mobile crisis teams)
Early Intervention	**B - Service Delivery & Technology**	Providing support and care at the first signs of distress to prevent severe or chronic conditions from developing
Digital Intervention	**B - Service Delivery & Technology**	The use of self-help websites and other technologies to promote well-being and provide psychoeducation

Community-Based Care	**B - Service Delivery & Technology**	Model of healthcare and social support that provides services within an individual's community (such as in their home, school, or local center) Focuses on patient-centered and personalized care to maintain independence and emphasise prevention and early intervention as well as the importance of ongoing care – especially in familiar environments
Workforce Expansion	**C - Workforce & Training**	Increasing numbers and diversity of trained professionals (psychiatrists, psychologists, social workers, counsellors)
Task-Sharing	**C - Workforce & Training**	AKA. Task-shifting The systematic delegation of tasks to less specialised workers to maximise the efficient use of resources and manpower Involves training mid and low level healthcare workers – such as clinical officers, and auxiliary nurses and, CHWs (community health workers) – to offer some services offered by specialised workers to optimise the span of the limited workforce.
Peer Support Specialists	**C - Workforce & Training**	Individuals with lived experience of mental health struggles who are trained to support others in their recovery journey.
Psychological First Aid (PFA)	**C - Workforce & Training**	Humane, supportive, and practical response to people suffering from severe crisis events. A key training component for frontline staff.
Mental Health Literacy	**D - Public Health & Prevention**	Public knowledge and understanding of mental health, causes, self-care, and resources for seeking help.
Stigma Reduction	**D - Public Health & Prevention**	Efforts to challenge negative attitudes and discrimination or ostracization associated with mental illness.
Protective Factors	**D - Public Health & Prevention**	Conditions or attributes that contribute to the likelihood of positive outcome of mental health. (Social support, resilience, coping skills, positive psychological attributes)
Social Determinants of Health (SDOC)	**D - Public Health & Prevention**	Non-medical factors (like housing, employment, education, and social isolation) that influences mental health outcomes, and require cross-sectoral policy action.

Country Stances

Country	Bloc	Unique Stance

Australia	*Bloc **A***	• **Systemic Reform**
		  ○ Pandemic triggered need for reform
		• **Understanding of increased and complex needs**
		• **Improving accessibility**
		• **Digital care**
		  ○ Ensuring services are **safe** and **effective**
		  ○ Social Distancing > Digital Mental Health Support
		• **Primary care integration**
		  ○ Stepped care
		• **Acknowledging Severe Workforce Shortages**
		  ○ Attracting and Training Mental Health workers
		• **Focus on High-Need Populations**
		  ○ Young people
		  ○ Individuals with Severe Mental Issues
		  ○ Individuals impacted by trauma
		
		• **The National Mental Health and Suicide Prevention Agreement**
		• **The National Mental Health and Wellbeing Pandemic Response Plan**
		• **The Better Access initiative**
		  ○ Ensuring **affordable** access to clinical treatment via General Practitioners (GPs)

Canada	*Bloc **A***	• **Canada Mental Health Transfer**
		  ○ Initial commitment of $4.5 over 5 years
		• **Broad Federal Funding for Health**
		  ○ $25 billion over 10 years (from 2023)
		  ○ Partially improving **access** to **quality** mental health and substance use services
		• **Wellness Together Canada (WTC)**
		  ○ **Free online** portal launched during the pandemic
		  ○ Offers: self assessment resources, short-term, one on one virtual sessions
		  ○ Shifts to make it permanently accessible — specifically in rural areas
		• **988 / Suicide Crisis Helpline**
		  ○ National 3-digit helpline
		  ■ Provides **urgent**, **bilingual**, 24/7 suicide prevention support
		  ○ Created with the acknowledgement of **increased demand** for mental health services post-pandemic
		• **Integration with Primary Care**
		  ○ Push from various mental health organizations and the government to better **integrate** mental health and substance use services
		• **Workforce Planning**

		○ Recognized need for **proportionate resources** ○ **Integrating** mental health providers into public health **insurance** ● **Targeted support for vulnerable populations** ○ Youth & Children ○ Indigenous Communities ○ Front-line and Healthcare workers ○ Individuals with substance abuse issues
China	*Bloc **B***	● **Focus on Students and Education** ○ **Special Action Plan (2023-2025)** ○ Goal : 95% of schools equipped with full or part-time mental health professional ● **Leveraging Digital/Virtual Care** ○ **Digital Mental Health Interventions (DMHIs)** ■ Mobile applications ■ WeChat mini-programs ■ Video conferencing ● Overcome logistical barriers — eg. **Rural areas** ● **Strengthening the Service System** ○ **Top-Down** : Led by a National Center for Mental Health — responsible for policy, guidelines, and training ○ **Bottom-Up (Community Integration)** ■ **Improving :** ● Primary care centers ● Community health service — screening high risk groups, providing initial evaluations and referrals ○ **Targeted Programs** ■ **686 Project** ● Management and treatment of severe mental illnesses ● **Struggles** ○ **Shortage** of professionals ○ **Uneven resource distribution** ■ The **imbalance** of infrastructure in **urban** and highly populated regions ○ Government **funding** ■ Comparatively **low per capita funding** > Unrealistic and unreasonable prices
Cuba	*Bloc **D***	● **Primary Care Integration** ○ **Community Based Care** ■ Utilizing **existing infrastructure** ■ Door-to-door **surveillance** ● **Identifying** and **following up** on physical or emotional issues and psychological distress ○ **Preventative Focus** ■ **Curative** treatments, **rehabilitation**, **social integration** ■ Aims to **spot** and **address** potential problems at a **community level** ● **Rapid Mobilization and Crisis Support**

		○ **National Action Protocol** ■ **Interdisciplinary** and **intersectoral** group updating a **protocol** that includes mental health care — especially in relation to the progression of the pandemic ○ **Expanded Telephone Counselling** ■ Free 24 hour national "psychosocial and mental health support line" ■ **Whatsapp Groups and Telecare** ● **Training and Social Engagement** ○ **Retraining** and **reinforcing training** for various mental health professionals as well as social workers ○ **University Student Mobilization** ■ Relies on **university students** to assist in carrying out door-to-door **procedures** ● **Material Shortages** ○ **Pharmalogical interventions** are limited due to the lack of essential supplies and drugs ● **Migration of Personnel** ○ **Specialized care** is limited due to the migration of healthcare professionals
France	*Bloc **A***	● **Direct Reimbursement for Psychological Care** ○ **Mon Soutien Psy (My Psy Support)** ■ Originated in 2022 ■ Covering **costs** of a **set number** of psychological **sessions** for citizens 3 years and older ■ **Direct booking** of psychological care without the need for GP referral (though it may be needed prior to the support being given) ● **Focus on Vulnerable Populations** ○ **Suicide Prevention** ■ **3114 emergency line** — fully implemented and actively being improved ■ Expansion of the **VigilanS** contact system — **follow up** for individuals that have attempted suicide ● Includes **special changes** for **younger people** ○ **Student Support** ■ **Vouchers for therapy (Chequés Psy)** ○ **Digital Outreach** ■ Partners with helplines like **Fil Santé Junes** (for 12-25 years old) ■ Providing **anonymous**, **free online support** and **referrals** ● **Investment and Integration** ○ **Recovery and Resilience Plan** ■ **Integrated Care** (GPs and Nurses as first-line support) ● Emphasizing **early detection** and better **local coordination** ■ **Infrastructure Renovation** ● **Renovation** and **Modernization** of medical centres and

		hospitals — including psychiatric units ■ **Recruitment** ● Enhance **benefits** of mental health **work** to secure more potential mental health professionals
Germany	*Bloc **A***	● **Combatting over-reliance on inpatient care** ○ Previously 50% of mental health funding was allocated to psychiatric hospitals > leads to an **imbalance** ● **Improving Sectoral Coordination** ○ Lack of **coordination** between **inpatient**, **day-clinics**, and **outpatient** (private practices) care > **incoherent** progress for individuals ● **Integrating Digital Care** ○ Lack of **digitized** care ● **Global Treatment Budget (GTB) Models** ○ Flexible and Integrative Treatment Models (**FIT64b**) > moving away from traditional daily and performance based costs, towards a **lump-sum budget** ■ Allows hospitals to **reinvest** saved budget from impatient care to flexible outreach home care and **integrated** in and outpatient units ● **Stepped-Care Models** ○ Aims to **optimize** treatment **allocation** and create **vacancy** in outpatient treatment by **reserving intensive care** for those at the highest risk ● **Investing in Digital Health** ○ **Digital Health Apps** ■ Certified health apps for mental well-being, sleep, relaxation and various psychological support ○ **Telemedicine** ■ **Telephone and Video Consultations** ● Integrated to improve **accessibility** during the pandemic and post-pandemic with the consideration of people with **anxiety** or anxiety related disorders ● **Comprehensive Mental Health Surveillance** ○ **Improves access especially in times of crisis**
India	*Bloc **B***	● **Digital Initiatives** ○ **National Tele Mental Health Programme (Tele-MANAS) — launched in 2022** ■ 24/7 Helpline ■ Offers **accessibility** and **confidentiality** ● Immediate support with **anonymity** — encourages **help-seeking behaviour** (especially in areas with severe **social stigma**) ■ **Tiered System of Care** ● **Level 1** : Immediate tele-counselling by trained professionals ● **Level 2** : Referral to specialist care through audio or

		video consultation ● **Level 3** : Referral for in-person services at nearby centers for complex cases ■ Operates **across** numerous **states** and union territories ■ Critical **high volume** part of infrastructure ● **Integration into Primary Care** ○ **Ayushman Bharat initiative** ■ **Health and Wellness Centres (HWCs)** — more than 160,000 primary care facilities are being upgraded to HWCs ■ **Grassroots Capacity** focus on mental health screenings, basic counselling, and referral services. Aims to make care accessible to communities and shifting tasks based on levels of speciality ● **District-Level Services** ○ **District Mental Health Programme (DMHP)** — part of the **National Mental Health Programme (NMHP)** is being expanded and further supported ■ **Community Services** : providing **counselling**, **outpatient** services, psycho-social **interventions**, and **support** for **severe** mental disorders at the **Community Health Centre (CHC)** and **Primary Health Centre (PHC)** levels. ○ **Suicide Prevention : The National Suicide Prevention Strategy (NSPS)** ■ Launched post pandemic ■ Emphasizes multi-sectoral actions ● **Establishing crisis helplines** ● **Promoting community awareness** ● **Mental health screenings in educational settings** ● **Addressing Human Resource Scarcity** ○ **Online training** : National Institute of Mental Health and Neuro-Sciences — created **online modules for training general health workers in psychosocial support (eg. IGOT-Diksha)** ○ **Academic Programs :** New courses are being introduced to increase the presence of **specialized** mental health **manpower**
Indonesia	*Bloc **C***	● **Screenings & Integration** ○ **Free Mental Health Screenings** ■ Planning on **free** mental health screening for all citizens (especially **children** and **adolescents**) ■ Using **community health centers** and **clinics** across the nation ■ Aims to be on a **mass scale** – surpassing previous campaigns ○ **Strengthening Primary Care** ■ **Integrating** mental health programs into community health centers ■ **Decentralizing** care is critical in a nation with only around **1,000 psychiatrists** ○ **Addressing Adolescent Mental Health** ■ Backed by the Indonesia-National Adolescent Mental Health Survey **(I-NAMHS, 2022)** → data showed significant numbers of adolescents experiencing mental health issues with very few

		reporting that they seek help. ● **Digital Health & Telemedicine** ○ **Online appointments** were necessary in order to overcome the challenges of **geographic barriers** and bypass the **stigma** associated with hospital visits. ○ Government Platforms such as **SEJIWA** – provided a **telephone hotline** and **family care** support during crises ○ **Digital Inclusion** is another roadblock, **rural areas** in Indonesia may lack access to technology making Digital Medicine inaccessible. Digital Literacy is also a consideration that must be made, with many rural populations lacking **exposure to technology** ● **Challenges** ○ **Minimal budget** (2-3% of the total health budget) has been allocated to mental health in the past, many professionals argue this percentage is insufficient ○ **Human Resource Shortage** is a major issue in Indonesia with an astonishing lack of specialists in mental health (psychologists, psychiatrists, and psychiatric nurses) ○ **Infrastructure Deficiencies** cause unequal and inadequate access with many provinces lacking psychiatric facilities as well as the missing integration of mental health services in general hospitals. ○ **Stigma & Pasung** ■ Indonesia continues to battle the ingrained social stigma and inhumane and illegal practices of Pasung. ● **Pasung** : the physical restraint or chaining of people with severe mental illness ■ **Awareness and rights-based care** is a necessary part of reforming mental health care post pandemic
Iran	*Bloc **D***	● **Adapting and Leveraging the existing Primary Health Care (PHC) network** ○ **Health Houses (rural)** & **Health Posts (urban)** – staffed by community health workers who provide **essential** care including **screenings** for mental health and **referrals**. ○ **Integration of Services** uses the **WHO's** Mental Health Gap Action Programme (**mhGAP**) protocols in order to handle **initial detection** and **long-term** follow ups. ○ Initiatives like the **SERAJ** Program are being tested and used to provide **preventative** treatment for mental health disorders and their risk factors across the population ● **Rapid Digital Transformation** ○ **National Helplines** were established promptly such as **4030**, **190** and **1666** to offer **psychosocial** support, **screenings**, and **consultations** ○ **Telemedicine** has been implemented in both **public** and **private institutions** – it was used to ensure a **continuation** of care for patients and to provide **new services** like **structured counselling** for grieving families and survivors of COVID-19 ○ **Electronic Health Systems** are integrated with digital care and allows for **consistent** patient **follow-ups** particularly for **severe** conditions and **vulnerable** populations

		● **Key Policies & Focuses** ○ **Targeted interventions** for mental health needs of **survivors** of covid and **grieving** individuals as well as those facing **distress** due to social restrictions ○ **Primary Health Care** staff were quickly trained to meet **increased** demand for **stress management** and **problem-solving** in times of crisis ○ **Suicide prevention** is being developed through the efforts to reduce **stigma** and **improve statistics** and increasing **access** to services as a **long-term priority**
Italy	*Bloc **A***	● **National Recovery and Resilience Plan (NRRP)** ○ As part of the **NextGenerationEU** program supported by the European Union, Italy is dedicating funds to : ■ Improving **resilience**, **accessibility**, **quality** of services, while including measures to **advance digitalization** and **community care** ■ **Psychologist Bonus (Bonus Psicologo)** ● Permanent contribution to help citizens afford private psychotherapy sessions → in response to the surge in demand following the pandemic ● **New National Mental Health Plan (2025-2030)** ○ **Integrated Care Model** ■ **Bio-psycho-social model** → moving beyond the traditional approach so that **social**, **environmental**, and **relational** factors can be considered as factors that influence mental health ■ **Primary Care Psychologist** ● Within community centers there will be **free**, **non-medicalized** support for **mild** to **moderate** mental health issues in order to **reduce** the **burden** on specialists and allow for **severe** mental health to have **priority** in specialist treatment ■ **Case Manager Role** ● For individuals with **severe** and **complex** mental disorders, a role to **personalize** and **coordinate** a therapeutic pathway to ensure **continuous support** ■ **Focus on Youth** ● Attention to **childhood** and **adolescence** is being given recognising the dramatic rise in psychological problems post-pandemic ● Aiming to create **regional integrated networks** for **diagnosis**, **treatment**, and **rehabilitation** of neuropsychic disorders ● **Service Delivery and Digital Medicine** ○ Expansion of **Telepsychology** is a continued necessity after COVID-19 ○ Surveys indicate **high satisfaction** from both **patients** and **professionals**

		○ **Community focus** are the backbones of the Italian system, with need for this to be reinforced as Italy navigates the **increase in patients** and **diversity of needs**
Japan	*Bloc **A***	● **Targeted support and suicide prevention** ○ New suicide prevention **guidelines** were made with emphasis on **women** and **young** people as populations disproportionately affected ○ **Youth focused education** is necessary but rarely implemented. For the first time in **40 years**, a course on **"prevention and recovery from mental illness**" has been added to high school health and physical education classes. ■ **Aims to encourage help seeking at an early age** ○ **Increased consultation access** ■ **Consultation services** expanded via phone, email, and social media ■ **Financial support** often received from the government ■ Offers **immediate psychological support** for COVID-19 related anxieties ● **Digital transformation** ○ **AI and Tele-counselling** are being utilized to offer **confidential** emotional support. ■ **AI apps include the Emol app** ■ This serves as a way for **objective** mental health tracking and **early detection** mechanisms ○ **Continuous wellness management is essential to achieving successful care** ■ **Digital platforms** may be used for **lifestyle guidance**, **medication reminders**, and **referral resources** ● **Structural reform** ○ Community based care is yet to be fully implemented however the pandemic exacerbated the need for services like **Assertive Community Treatment (ACT)** teams and **home-visit nursing care** ○ **Inpatient care** is being transformed to **reduce** average **inpatient stays** to align with **international standards** ○ **Promoting peer support is necessary to creating systemic changes** ■ Providing **assistance** from individuals with **lived experience** of mental illness
New Zealand	*Bloc **A***	
Nigeria	*Bloc **C***	● **Legislative modernization** ○ Signed a new Mental Health Bill into Law in January 2023 – replacing the outdated Lunacy Act of 1958 ○ The law intends to be non-discriminatory, with human rights staying front of mind ○ Unfortunately, passing the law is only a first step. To successfully implement the law there would need to be a drastic increase in funding, human resources, and political will

		• **Infrastructure deficiencies**
		  ○ Nigeria has less than 350 trained psychiatrists (causing a ratio far below the recommendation from WHO)
		  ○ Public facilities are extremely limited with only 9 federal psychiatric hospitals and 5 state psychiatric hospitals – all of which were concentrated in urban areas leaving millions with little to no access
		  ○ Underfunding remains a consistent issue in many countries, Nigeria is no exception with only 3-4% of the health budget being allocated towards mental health. Estimates state that 80% of individuals with serious mental health needs are unable to access care
		  ○ Health insurance is lacking and rarely covers the costs of psychological treatment – making it inaccessible for the majority of the population
		• **Policy direction**
		  ○ Task sharing approaches are becoming increasingly popular amongst rural populations due to the lack of specialists
		  ■ This involves training community and village health workers to provide basic support, assessments, and referrals at a primary care level
		  ○ The need for tele-psychiatry has been emphasized by the pandemic. In Nigeria, non governmental groups (NGOs) and private groups are pushing for this change using social media to reach the public eye.
		• **The role of non-governmental organizations (NGOs)**
		  ○ Awareness and advocacy are necessary steps carried out by NGOs, with an urgent need for reduced social stigma around spiritual causes leading to traditional healers attempting to fix a possible genetic issue
		  ○ Critical response is necessary in times of crisis, organizations like Mentally Aware Nigeria Initiatives (MANI) reported 70-150% increase in clients indicating the necessity for NGOs especially in times of crisis

Norway	*Bloc **A***	• **The escalation plan**
		  ○ Financial commitments include an estimated 3 billion norwegian krones over a 10 year period (2023-2033)
		  ○ 3 main areas of focus include :
		  ■ Health promotive and preventative measures
		  ■ Quality and accessible care
		  ■ Services for long-term, complex needs
		• **Community services**
		  ○ Municipal focus ensures accessible care within communities
		  ○ Early intervention is targeted to vulnerable groups (such as the youth) to identify mental health issues and substance abuse
		  ○ School-based support is being recognised as essential especially with the lasting impacts of the pandemic on children and adolescents
		• **Digitalization and Telehealth**
		  ○ National portal : Helsenorge.no are being used to provide remote care and self management resources such as apps and videos
		  ○ E-Consultations are becoming more common as the pandemic highlighted the need for remote care options
		
		• **Context**
		  ○ High professional ratio : one of the highest per capita ratio of

		psychiatrist, psychologists, and mental health personnel ○ Free and accessible care especially for children is a priority for Norway ○ Integrated care is already implemented with primary care related to mental health handled by GPs and municipal nurses while more intensive care is left to the specialists
Philippines	*Bloc **C***	● **Universal healthcare integration** ○ The department of health is actively attempting to **integrate** mental healthcare into primary care through the **Mental Health Gap Action Programme (mhGAP)** ■ This will train non-specialists and non-health professionals to handle common mental disorders ○ **Specialty centers are a part of the Philippine Health Facility Development Plan (2020-2040)** ■ Notably **funding** has been prioritized for specialties such as neonatal, heart, and kidney care ○ **PhilHealth Coverage** provides benefits for outpatient services including; **consultations**, **psychoeducation**, and **psychosocial support** at certified mental health facilities ● **Policy framework** ○ **Strategic framework (2024-2028)** ■ In collaboration with the **WHO,** the **PCMH** launched a 5 year framework to give guidance with **policies** and **programs**
Russia	*Bloc **C***	● **Outpatient care** and **telepsychiatry** is being **promoted** to maintain access during **social distancing** and in times of **emergency** ● Broader **health initiatives** to aid in **integrating mental health** care into **primary care** when possible ● Prior to the pandemic **efforts** were being made to : ○ **Decrease** the presence of **over-centralized psychiatric** hospitals → (One of the **highest psychiatric beds per capita** in Europe) ○ Focusing on **community-based care** ○ Implement **psychosocial rehabilitation** ○ Integrate **mental health care** into **primary care** ● **Context :** ○ Covid 19 **pandemic** alongside the Russia-Ukraine **war** heightened **stressors** (such as; **increased anxiety** and **depression** levels) – particularly among **students** and **adolescents**) ○ Pandemic **encouraged transition** to **remote** and **telehealth** services for providing mental health care ○ **Psychiatric wards** struggled to organize Covid 19 **treatment** → led to the need for **quarantine** for residents and employees ○ **Community Based Care** is **extremely limited** ○ **High** number of **psychiatrists** – **shortage** of **social workers** and other **multidisciplinary specialists**) ○ **Challenges** : High presence of **stigmas** – due to **low mental health literacy** ○ Inadequate **public funding** ○ Negative **reforms** (“optimization”)

		○ Need for **telehealth adoption** ○ Needs **focus** on **vulnerable groups**
Singapore	*Bloc **A***	● **Infrastructure** project : ○ **Alexandra Hospital** → includes **inpatient psychiatric beds** and **rehabilitation facilities** ● **Workforce initiatives** : ○ Aims for an **increase** of **30-40%** in **psychiatrists** and **psychologists** ○ Aims to **train** over **100,000 frontline personnel** (including : **teachers**, **social workers**, and **general practitioners/GPs**) for **early intervention** and **support** ● **Access** : ○ **Expansion** of **mental health services** in all **polyclinics** (a single healthcare facility that offers a range of services) and integrated with **general practitioners** (GPs) ■ Under the **Healthier SG preventive care strategy** by **2030** ● **Use** of **tiered care model** (or stepped care model) ● **Use** of **"No Wrong Door Approach"** → ensures **access** to **immediate services** regardless of point of entry ○ National **mental health helpline**, mindline "**1771**", or **community outreach teams** ● **Development** of **mental health education curriculum** for all **pre-university students** to boost **literacy** and **coping** strategies ● **Development** of **public campaigns** like "**It's OKAY to Reach Out**" → used to **normalize conversations** and **encourage help-seeking behaviour** ● **Tripartite Advisory** : ○ **Government** issued an **advisory** to **employers** recommending measures such as: **flexible work arrangements** (**FWAs**), designating **Workplace Mental Well-Being Champions**, and **providing access** to **counseling** services ● **Anti-discrimination legislation** such as : **New Workplace Fairness** will **prohibit discrimination** on the basis of mental health conditions
South Korea	*Bloc **B***	● **Mass Counseling :** ○ Aims to provide **psychological counseling** to **one million** people by **2027** ○ **Prioritizing medium** and **high-risk groups** ● **Screening** : ○ **Mental health checkups** for **young people** (**20-34**) increased from **once** every **10** years to **once** every **2** years ■ For **early intervention** on **depression**, **schizophrenia**, and **bipolar disorder** ● **Public Education** : ○ **Mandatory suicide-prevention education** introduced **annually** for a large proportion of the population (**targetting 16 million** people) – **including students** and **employees** ● **Emergency Infrastructure :** ○ Aims to **double number** of **acute care beds** for **psychiatric patients nationwide**

		● **Joint Response Teams :** ○ **24-hour joint response centers** – including **mental health specialists** and **police officers** ○ Establishing **emergency dispatch centers** for **efficient access** to **emergency care** ● **Integrated Hotline** : ○ **Suicide hotline integrated** with other **counseling** services into a **single number** (**109**) ○ **Increase** in **counselors** ● Expanding **mental rehabilitation facilities** and **strengthening continuous treatment** and **case management** (for **patients** with **severe mental illnesses**)
Spain	*Bloc **A***	● **Recruitment** : ○ Aims to significantly **increase** the number of **specialized professionals** → to **reduce waiting times** and **increase access** ● **New Specialization** : ○ The creation of the "**Child and Adolescent Psychiatry**" specialty → in response to the pandemics **disproportionate impact** on **youth** ● **Suicide Prevention** : ○ Establishing a **24/7** national suicide prevention **helpline** (024, "Call for Life") and the development of specific suicide prevention **action plans** ● **Primary Care Integration** : includes; ○ **Strengthening care** at **all** levels ○ Emphasizing **primary care** – to **prevent** the need for **specialist** intervention ○ **Early** intervention ● **Decentralization** : ○ Plans for a **community-based care model** (that favors **recovery** and **social integration**) ● **Human Rights and Stigma** : ○ Eliminating **treatments** that are **non-consensual** or include the use of **mechanical restraints** ● **Commissioner** : ○ Appointed a commissioner for mental health – **responsible** for **ensuring** mental health is **considered** and **addressed** across **all public policies**
Sweden	*Bloc **A***	● New **10 year** strategy ● **Whole-Of-Society Approach :** ○ Mandates 27 **authorities** to **implement** mental health **considerations** in **policies** and **varying contexts** ● **Targeted Investment :** ○ (equivalent of) **165 million USD** dedicated to **mental health** and **suicide prevention** ● **Research :** ○ National **research programme** for mental health (developed in 2021) is **supporting research** on ■ **Effective psychosocial interventions**

		■ **Preventative measures** ● **National Hotline :** ○ Enhancing and coordinating **national services** ○ **Knowledge building** around suicide prevention ● **Primary Care Integration :** ○ Must clearly **provide services** for **common mental health needs** → to achieve **equal, person-centered,** and **accessible care** ● **Vulnerable Groups :** ○ Attention to **improving** and **coordinating** efforts for **children's** and **young people's** mental health – of which was **heavily impacted** by the **pandemic**
Switzerland	*Bloc **A***	● **Cantonal Autonomy :** ○ Leads to regional **variations** in : ■ **Access** ■ **Service models** ■ **Professional shortages** ● **High Outpatient Capacity :** ○ High **capacity** for **outpatient treatment** ○ High **professional density** ■ **Access** remains an **issue** since care is **concentrated** in larger cities ● **Universal Coverage :** ○ **Mandatory** basic health **insurance** ■ Ensures access to **outpatient** and **inpatient psychiatric treatments** ■ Premiums and co-payments may still be a burden ● **Increased Stress :** ○ Studies showed a **significant increase** in **stress** and **depressive symptoms** due to the **pandemic** ● **Youth :** ○ **Impact** of the pandemic on **youth** was **disproportionate** and requires **special attention** ● **Telehealth Acceleration :** ○ Pandemic **forced** a shift to **digital healthcare** ■ Accelerating **acceptance** and **billing options** for remote/digital psychiatric psychological consultations ● **Community-Based Care :** ○ Using **community-based care** to **promote early intervention** and **socio-psychiatric support** within the local community ● **Funding :** ○ Lack of **regulated funding bases** for **intermediate** services (such as : day clinics, home treatments, or case management) that **bridge the gap** between **inpatient** and **outpatient care**
Taiwan	*Bloc **B***	● **Community Support :** ○ Moving away from **long-term hospitalization** ■ Establishing **community-based mental health care centers** → **diversifying** community support **services**

		● **Stigma :** ○ Goal to **ensure** individuals with mental illnesses can achieve **equal living** with others ● **Crisis Management :** ○ Establishing a **mechanism** for **handling** mental health **emergencies** → improving **cross-agency cooperation** for **community care** and **crisis intervention** ● **Whole-Of-Society Approach :** ○ Mandating **mental health** in all **policies** ○ Requires 13 **ministries** and **agencies** to **integrate** mental health considerations into their **planning** ● **Primary Prevention :** ○ Initiating **age** and **population-specific** mental health **promotion programs** ● **Targeted Investment :** ○ **Subsidies** created for **specific issues** like alcoholism treatment or internet addiction intervention **programmes** ● **Human Rights Alignment :** ○ National **guidelines** were heavily influenced by the **UN convention** on the rights of persons with disabilities (**CRPD**) ○ Focus on **enhancing** patient **rights** especially around **involuntary hospitalization** and **treatment decisions** ● **Digitalization :** ○ Strengthening **technological** and **digital** mental health **infrastructure** ○ Aiming to **conduct** mental health **surveys**, integrated **data**, and **enhance** care **effectiveness** ● **Workforce :** ○ (Equivalent of) $101 million USD in national health insurance **budget** dedicated to: ■ **Long-acting injectable psychiatric medications** ■ **Indirectly support service capacity** ■ **Funding to strengthen the professional workforce**
Thailand	*Bloc **B***	● Decentralization ○ Even prior to the pandemic, Thailand was moving towards mental health care services being offered in general hospitals, community care centers, and community hospitals ● Local Networks ○ The department of mental health relies heavily on village healtj volunteers and collaborations with non-health networks like schools, temples, and community leaders ● No Stand-Alone Centers ○ Integrating mental health services into existing healthcare facilities instead of establishing independent psychiatric facilities ○ Aims to improve accessibility ● Shortages ○ There are an insufficient number (and poor geographical distribution) of specialised staff ○ Professionals mainly concentrated in major cities like Bangkok

		● Non-Specialists ○ Training general practitioners (GPs) to become sub-specialists in preventative medicine ○ Focusing on community mental health ● Pandemic ○ Focused on the consequences of the pandemic ■ Rising suicide rates ■ Increased prevalence of depression, anxiety, or stress ● Policy Focus ○ Focus on improving access to mental health and drug rehabilitation services ● Digital Tools ○ Using teleconsultation
United Kingdom	*Bloc **A***	
United States	*Bloc **A***	
Venezuela	*Bloc **C***	

Blocs

Bloc A :

- **Australia**
- **Canada**
- France
- Germany
- Japan
- New Zealand
- **Norway**
- Singapore
- Spain
- **Sweden**
- **Switzerland**
- United Kingdom
- United States

Bloc B :

Bloc Bi :

- China
- India
- South Korea

Bloc Bii

- Indonesia
- Nigeria
- Philippines
- Russia

Bloc Biii

- Cuba
- Iran

WHO Stance

- Guide 1
- Guide 2
- Guide 3
- Guide 4
- Guide 5
- Atlas

Mental health system governance

- **Policies and plans:** 81% of responding countries reported having distinct policies or plans for mental health; slightly over a half (56%) reported that they had distinct or integrated mental health plans or policies for children and adolescents, and 42% reported one for older adults. Most of the responding countries reported that they largely (85%) or fully (72%) complied with human rights instruments (as measured on a five-point checklist); however, only 50% reported that they were both fully compliant with human rights and also had the appropriate resources to implement the national mental health policy or plan effectively.

- **Legislation:** 72% of the 144 responding countries reported that they had a distinct law for mental health. Of 98 countries that completed a checklist on compliance of their laws with international human rights instruments, 73% were partially compliant and only 45% fully compliant.

- **Collaboration:** A high, increasing level of collaboration between governmental and nongovernmental stakeholders was observed, with 79% of responding countries reporting functional collaboration with nongovernmental organizations (NGOs).

Mental health financing and workforce

- **Financial protection:** Four of five responding countries (81%) reported that mental health care and treatment were included in a publicly funded health or financial protection scheme. In at least 50% of the responding countries, there is little or nothing to pay (≤ 5%) towards the cost of mental health inpatient care, outpatient care and psychotropic medication. In some countries, however, people with mental health conditions contribute mostly or entirely to their care (> 50%); for example, 28% of responding countries reported a high contribution for psychological therapy. Major discrepancies were observed among income groups.

- **Expenditure:** Data from 75 responding countries showed that the global median expenditure on mental health was US$ 2.69 per capita, with wide variation among country income groups, from < US$ 1 in low-income (US$ 0.04 per capita) and lower–middle-income countries (LMIC) (US$ 0.34 per capita) to US$ 65.89 in high-income countries (HIC). The global median percentage of total government health expenditure allocated to mental health is 2.1%, which is the same as that derived and reported in both 2017 and 2020, ranging from ≤ 1.5% in LMIC to 4.3% in HIC.

- **Workforce:** The global median number of specialized mental health workers is 13.5 per 100 000 population, ranging from 1.1–2.4 in LIC and LMIC to 67.2 in HIC. A median of 1.5 specialized mental health workers was reported per 100 000 children and adolescents. Mental health nurses comprise the single largest proportion of the specialized mental health workforce (globally, 43% of the total), followed by psychologists (22%) and psychiatrists (16%).

Mental health services

- **Service reform:** Globally, mental health service reform is progressing slowly: less than 10% of responding countries have fully moved from institutional-based models of care to community-based care. Most (52.9%) remain in the early stage of transition, with more beds and services still concentrated in psychiatric hospitals.
- **Integration of mental health into primary care:** Of the 138 responding countries, most (74%) met three of the five criteria for functional integration of mental health into primary care; 32% met four, and only 12% met all five criteria.
- **Telehealth services:** 63% of the responding countries declared that telehealth services were available and were being used. In tele mental health, services are delivered by health-care professionals using information and communication technologies to exchange valid information for diagnosis, treatment and prevention of disease and injuries.
- **Social support:** Globally, a large majority (88%) of responding countries reported that people with mental health conditions benefited to some extent from social support (e.g. disability payments or income support), and over half (59%) reported that most people with severe mental health conditions received such support. The proportion differed widely among WHO regions and country income groups.
- **Emergencies:** 65% of responding countries reported having a system for MHPSS for emergency preparedness and/ or disaster risk management (a large increase from 45% in the Mental Health Atlas 2020). An even higher percentage (81%) reported that mental health and psychosocial support were provided as part of their emergency response.
- **Outpatient care:** The global median of 0.45 facilities per 100 000 population is underlied by wide differences, from more than 1 facility in upper–middle-income countries (UMIC) and more than 2 in HIC, to fewer than 0.1 in LIC and LMIC. The proportion of responding countries that offer outpatient services in community-based facilities also differed widely, from 25% in LIC and 50% in LMIC to 76–80% in UMIC and HIC.
- **Inpatient care:** Globally, a median of one mental health inpatient and residential care bed per 10 000 population was reported; most inpatient beds are in specialized psychiatric hospitals (62%). Data from 45 countries indicate that 49% of all admissions to psychiatric hospitals and psychiatric units in general hospitals are involuntary. In more than one in five admissions to psychiatric hospitals, the length of stay is > 1 year.
- **Service coverage:** Based on a small sample of 22 countries that provided complete data, service coverage for psychosis was estimated to be 40%, ranging from < 10% in LIC and > 50% in UMIC and HIC.

Mental health promotion and prevention programmes

- Mental health promotion and protection: 63% of responding countries reported at least two functional mental health promotion and protection programmes. Most of the programmes addressed early childhood development (86% of responding countries), suicide prevention (80%) and school mental health (78%).
- Suicide prevention: Nearly half of 138 responding countries (47%) reported having a distinct or integrated national suicide prevention strategy, policy or plan and a suicide prevention programme. In a smaller sample, of 54 responding countries, 80% were found to have a well-functioning suicide prevention programme.

Most of the responding countries considered that their laws promoted at least one of the following standards:

- transition towards community-based mental health services (74 countries, 73% of responding countries);
- promotion of the rights of people with mental health conditions and psychosocial disabilities to exercise their legal capacity (68 countries, 68%);
- informed consent to treatment (77 countries, 79%);
- promotion of voluntary admission (74 countries, 77%);
- prevention of seclusion and constraint (62 countries, 65%);
- provision for procedures to enable people with mental health conditions and psychosocial disabilities to protect their rights and file appeals and complaints to an independent legal body (74 countries, 75%); and
- provision for regular inspections of human rights conditions in mental health facilities by an independent body (57 countries, 58%).

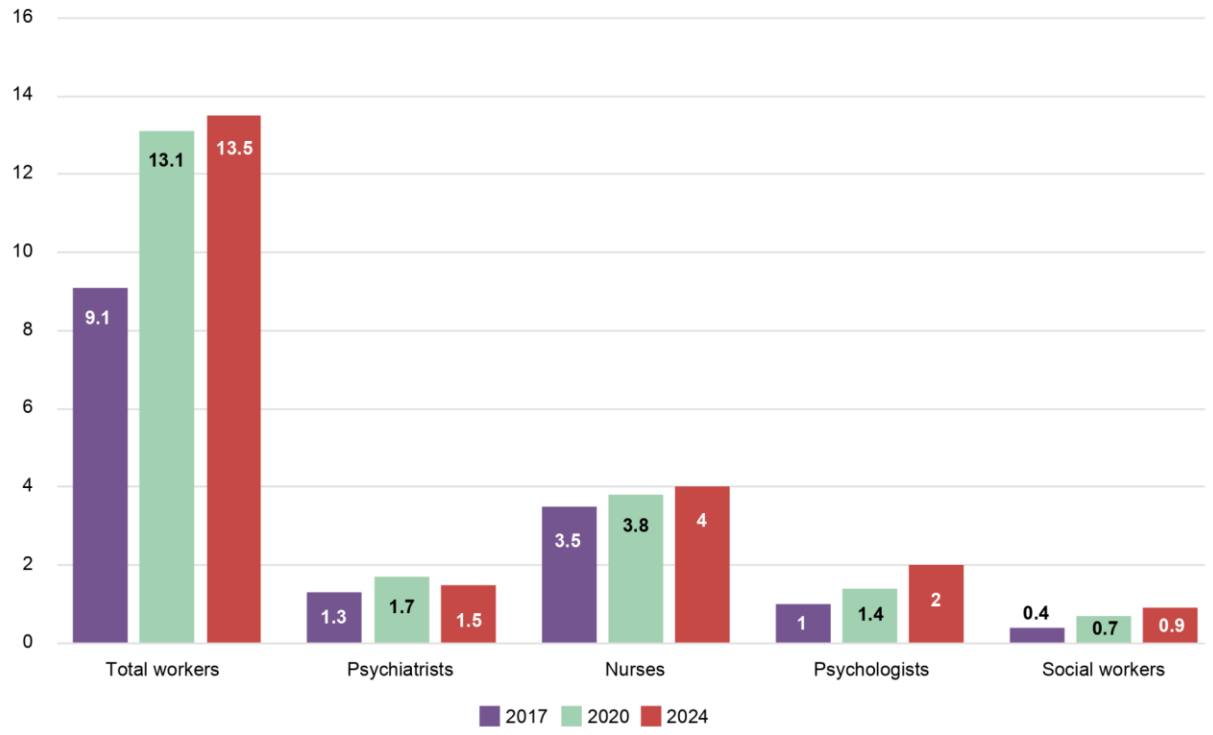
Fig. 13 provides a breakdown by World Bank income group of compliance with the standards. All HIC complied with two of the seven standards, and at least 80% of HIC and UMIC complied with several standards. By contrast, compliance rates among LIC were less than 40% for each standard. It should be noted with respect to this question that self-reported data may result in overestimation of compliance.

No. of specialized mental health workers for adults (per 100 000 population)	Government mental health workers	Nongovernment mental health workers	All mental health workers
Globally (n=132)	9.6	1.5	13.5
By World Bank income group			
LIC (n=18)	0.9	0.3	1.1
LMIC (n=35)	1.9	0.4	2.4
UMIC (n=38)	13.6	4.0	19.3
HIC (n=41)	46.6	13.7	67.2
By WHO region			
AFR (n=33)	1.9	0.3	2.2
AMR (n=28)	13.4	8.7	22.2
SEAR (n=7)	2.3	1.6	4.0
EUR (n=27)	52.0	11.3	80.4
EMR (n=19)	3.7	1.9	4.7
WPR (n=18)	12.8	3.9	14.1

Table 20. Numbers of mental health workers globally, by occupational category, World Bank income group and WHO region (median per 100 000 population)

	Psychiatrists	Mental health nurses	Psychologists	Social workers	Occupational therapists	Other specialized mental health workers	Peer-support workers
Globally (n=132)	1.5	4.1	2.0	0.9	0.2	0.8	0.0
By World Bank income group							
LIC (n=18)	0.1	0.3	0.1	0.1	0.0	0.1	0.0
LMIC (n=35)	0.3	0.7	0.3	0.2	0.0	0.2	0.0
UMIC (n=38)	1.7	5.4	4.0	1.2	0.4	1.5	0.0
HIC (n=41)	7.0	27.0	9.0	4.1	2.8	2.7	0.0
By WHO region							
AFR (n=33)	0.1	0.6	0.2	0.1	0.0	0.1	0.0
AMR (n=28)	1.7	3.6	6.4	1.7	0.2	1.5	0.0
SEAR (n=7)	0.5	0.4	0.2	0.3	0.3	0.2	2.6
EUR (n=27)	9.9	28.4	9.3	3.6	4.4	1.9	0.4
EMR (n=19)	1.3	3.2	1.0	0.4	0.1	0.2	0.1
WPR (n=18)	2.2	12.9	0.4	0.2	0.0	1.9	0.0

Figure 26. Mental health workforce, 2017–2024 (median number per 100 000 population)



- guidelines for mental health integrated into primary health care available and adopted nationally;
- pharmacological interventions for mental health conditions available and provided in primary care;
- psychosocial interventions available and provided at primary care level;
- health workers in primary care trained in management of mental health conditions; and
- mental health specialists involved in training and supervising primary care professionals.

Term	*Group*	*Description*
Psychiatric hospital (or mental hospital)	***Facilities***	Provides **inpatient care** and **long-term stay services** for people with mental disorders **Private** and **public** institutions involved **Excludes** community units, substance abuse treatment facilities, and solely intellectual disabilities treatment facilities For **any** age group
Psychiatric unit in a general hospital	***Facilities***	**Weeks** to **months** (stay duration) **Excludes** mental hospitals, residential facilities, substance abuse treatment facilities, and solely intellectual disabilities treatment facilities For **any** age group
Mental Health Community Residential Facility	***Facilities***	**Non hospital** facility Provides **overnight stays** for mentally ill people Both **private** and **public** (for and not for profit) **Staffed** or **unstaffed** **Homes**, **hostels**, **halfway houses**, and **therapeutic communities**
Mental Health Outpatient Facility	***Facilities***	**Manages** mental disorders and **related clinical and social issues** Both **private** and **public** (for and not for profit) Mental health **centers**, **outpatient clinics**, and **departments** in mental hospitals or general hospitals
Primary Healthcare Centre	***Facilities***	**First point** of entry/contact when receiving healthcare **Provides initial assessment** and **treatment** for **common health**

		conditions **Refers** patients to respective **specialists** for specialized care

Term	*Group*	*Description*
Nurse	***Personnel***	**Health professional** who has recieved an accredited **diploma** or **degree** in nursing
Occupational Therapist	***Personnel***	**Health professional** who has completed **formal** occupational therapy **training** and has an accredited **diploma** or **degree**
Other Health or Specialized Mental Health Worker	***Personnel***	**Health Professional** who has recieved **some level of training** but has not completed an accredited course to receive a degree or diploma Eg. primary care worker, psychosocial counselors and auxiliary staff
Peer-Support Worker	***Personnel***	**An individual** with **lived experience** of mental health issues**,** who has been **trained** to **provide support** and **care** for **people** with **mental health conditions**
Psychiatrist	***Personnel***	**Health professional** with **at least 2 years** of **post-graduate training** in **psychiatry** May have **training** in a **sub-specialty** of **psychiatry**
Psychologist	***Personnel***	**Health professional** who has completed **formal training** in **psychology** and received a **recognised degree** or **diploma**
Social Worker	***Personnel***	**Professional** who has completed **formal training** in **social work** and received a **recognised degree** or **diploma**

Term	*Group*	*Description*
Legal Capacity	***Other***	People with mental disabilities have the right to exercise their legal capacity and make decisions and choices on all aspects of their lives – **on an equal basis with others** **Supported Decision Making** exists which enabled a **mentally disabled** person to **nominate** a **trusted individual** to **consult** and **discuss issues** that may be affecting them
Mental Health and Psychosocial Support Minimum Service Package (MHPSS MSP)	***Other***	**Outlines** a set of **activities** that are considered to be of the **highest priority** for meeting the **immediate critical needs** of **populations** in **emergencies** **Based on guidelines, evidence, and expert consensus**
Mental Health Condition	***Other***	**Broad term** Covers **mental disorders** and **psychosocial disabilities** **Includes** other states such as; **significant distress, impairment of functioning,** or **risk of self-harm**
Mental Disorder	***Other***	**A syndrome characterized by cognition, emotional regulation, or behaviour that reflects a dysfunction in the psychological, biological, or developmental processes** **Usually associated with distress or impairment in: personal, family, social, educational, occupational, or other important areas of functioning** **(ICD-11)**
Publicly Funded Health or Financial Protection Scheme	***Other***	**System** to insure a **national population** against the **costs of**

		health care **Achieved** when **direct payments** made for the **exchange** of **health services** do not **expose** people to **financial hardship** nor **threaten** their **living standards**
Pharmacological Intervention for Mental Health Conditions	***Other***	**Use** of **psychotropic medicines** to reduce the **symptoms of mental health conditions** and **improve functioning** Listed on the **WHO Model List of Essential Medicines** – defines the **minimum medicine requirements for a basic health system**
Psychosis	***Other***	**Characterized** by **distorted thoughts** and **perceptions** and **disturbed emotions** and **behavior** **May include incoherent** or **irrelevant speech** Symptoms such as **hallucinations**, **delusions**, **severe behavioural abnormalities**, **disturbances** of **emotion** may occur
Psychosocial Interventions for Mental Health Conditions	***Other***	**Interpersonal** or **informational activities**, **techniques** or **strategies** for **biological, behavioral, cognitive, emotional, interpersonal, social, or environmental factors for improving health, functioning, and well being** Applied to : **psychoeducation, psychotherapy, counselling,** and other **non-pharmacological interventions**
Seclusion and Restraints	***Other***	**Seclusion** is the **voluntary placement** of an **individual – alone** in a **locked room** or **secured area** from which they are **physically prevented from leaving**

		Restraint is the **use** of a **mechanical device** or **medication** to **prevent a person from moving their body** **Alternatives** include **prompt assessment** and **rapid intervention** in **crises** with **problem-solving methods** and/or **stress management techniques** such as **breathing exercises**
Tele-Mental Health Services, Telemedicine Services	***Other***	**Using technology** to **deliver health-care services** when **distance** is a **challenge** For **diagnosis, treatment and prevention of disease** and **injuries, research** and **evaluation,** and for **continuing education** of **healthcare workers** to **improve** the **health** of **individuals** and **communities**

36% conducted a national survey for mental health in the past 10 years
58% reported having a nationwide digital health record system that includes mental health data

Target 1.1

- **80%** of countries will have **developed a policy or plan for mental health** that **aligns with international human rights guidelines – by 2030**
    - **2024 :**
        - **72%** responding countries **(+5% over 4 years)**
        - **63%** member states **(+3% over 4 years)**
- **Indicator 1.1**
    - **Existence** of a **national policy** or **plan** for **mental health** that is being **implemented** and is in line with **international human rights guidelines**
        - **2024 :**
            - **50%** of responding countries **(+26% over 4 years)**
- **Indicator 1.2**
    - **Existence** of a **national law** on **mental health** that is being **implemented** and is in line with **international human rights guidelines**
        - **2024 :**
            - **27%** of responding countries **(-8% over 4 years)**

Target 1.2

- **80%** of countries will have **developed or updated their law for mental health in line with international human rights equipment – by 2030**
    - **2024 :**
        - **43%** responding countries **(-4% over 4 years)**
        - **48%** member states **(-1% over 4 years)**

Target 2.1

- **50% increase** in **service coverage** for mental health by – **by 2030**
    - **Indicator 2.1.1**
        - **Proportion of people** with **psychosis** who **used services – in the past 12 months**
            - **2024 :**
                - **40%** worldwide **(+11% over 4 years)**
        - **Proportion of people** with **depression** who **used services – in the past 12 months**
            - **2024 :**
                - **9.1%** worldwide

Target 2.2

- **80%** of countries will have **twice** the number of **community health facilities – by 2030**
    - **Indicator 2.2**
        - Number of **community based mental health facilities**

- **0.64** per 100,000
- **0.52** per 100,000

Target 2.3

- **80%** of countries will have **mental health integrated into primary healthcare – by 2030**
  - **2024 :**
    - **34%** responding countries **(+3% over 4 years)**
    - **24%** member states **(-1% over 4 years)**
- **Indicator 2.3**
  - **A system** is **available** for the **integration** of **mental health** into **primary healthcare**
    - **2024 :**
      - **34%** of responding countries **(+3% over 4 years)**

Target 3.1

- **80%** of countries will have **at least 2 functioning national**, **multisectoral** mental health **promotion** and **prevention programmes – by 2030**
  - **2024 :**
    - **63%** of responding countries **(-5% over 4 years)**
    - **20%** of member states **(-32% over 4 years)**
- **Indicator 3.1**
  - **Functioning programmes** for **multisectoral mental health promotion** and **prevention** are **available**
    - **2024 :**
      - **63%** of responding countries **(-5% over 4 years)**

Target 3.2

- **33% reduction** in **suicide rates – by 2030**
- **Indicator 3.2**
  - **Suicide mortality rate (per 100,000)**
    - **2021**
      - **8.9** per 100,000 **(-0.1 over 3 years)**

Target 3.3

- **80%** of countries will have a **system for mental health and psychosocial preparedness for emergencies and/or disasters – by 2030**
  - **2024 :**
    - **65%** of responding countries **(+20% over 4 years)**
    - **48%** of member states **(+20% over 4 years)**
- **Indicator 3.3**
  - **A system for mental health and psychosocial preparedness for emergencies and/or disasters**
    - **2024 :**

- **65%** of responding countries **(+20% over 4 years)**

Target 4.1

- **80%** of countries will **routinely collect and report at least a core set of mental health indicators – every 2 years in their national information systems – by 2030**
  - **2024 :**
    - **44%** of responding countries **(+9% over 4 years)**
    - **32%** of member states **(no change over 4 years)**
- **Indicator 4.1**
  - **Core set** of **identified, agreed mental health indicators** routinely **collected** and **reported – every 2 years**
    - **2024 :**
      - **44%** of responding countries **(+9% over 4 years)**

Target 4.2

- **Twice** the amount of **global research** on **mental health – by 2030**
  - **2024 :**
    - **63%** of responding countries **(-5% over 4 years)**
    - **20%** of member states **(-32% over 4 years)**
- **Indicator 4.2**
  - **Number of published articles on mental health research**
    - **2024 :**
      - **68.590** published articles **(+3944 after 4 years)**

New Zealand

- **Mental Health Infrastructure Programme** part of **Infrastructure and Investment Group**
- **16 mental health** and **addiction** infrastructure **projects**
- Completed projects
    - the **Manawai individualised service units** at the **Rātonga-Rua-O-Porirua Mental Health Campus**
    - **Tiaho Mai** at **Middlemore Hospital**
    - Two new buildings at **Hillmorton Hospital.**
        - These house the **specialist mental health services** formerly located at the Princess Margaret Hospital
- The Programme bases its designs on the **best-practice Australasian Health Facility Guidelines.**
- Māori design features include **culture**, **meeting houses** and **dining halls** — to enable **hospitality**, **welcoming** and **settlement** for arriving patients. These elements also allow service users to **maintain** a **connection** with their **families**.
- New facilities aim to **group people together** based on **needs**
- Aims to create **facilities** that can **accommodate high**, **medium**, and **low patient needs**
- Resources are relatively **well spread out** in New Zealand
- **20%** of **New Zealanders** living with **mental health conditions can and do recover**. But that depends on a system that's **planned, staffed, and resourced** to meet people where they are
- **The RANZCP is calling for:** **("No Lifeline for Struggling Mental Health System in Budget, Say Psychiatrists | RANZCP")**
    - Dedicated **funding** for **specialist mental health**, protected from being **absorbed into other initiatives or general health budgets**
    - Investment to increase the **number** of **training places** for **psychiatry registrars** by **10% annually** for the **next five years**
    - **Support** to **recruit** and **retain New Zealand-trained psychiatrists** in **Health NZ**
- **$28 million over four years** to fund the transition from a **Police-led response** to a **mental health response to 111 mental health distress calls, including associated funding for stage one psychiatry registrars** - but workforce **shortages** could limit its **impact**, and without greater support for specialist services, more people will end up in distress.

Economic & Social Factors

- **Increase** in **demand** → **higher** treatment **costs**
- **Direct** and **indirect** costs have risen
    - **Direct** : consultations, pharmacological treatment (medication), and hospitalizations
    - **Indirect** : reduced productivity, sick leave, disabilities related to mental health
- **Digital medicine** though present remain **costly**
- Pandemic **increased** the need for the use of **health** and **social benefits** (e.g. sick leave, disability benefits)
    - Due to this **social insurance budgets** are being **strained**
- **Work incapacity** has caused a sudden rise in **payouts**
- After being ill, significant numbers of individuals **could not** return to their **full potential**
- Specific **conditions** caused by **factors** of the **pandemic** like :
    - **Prolonged social isolation**
    - **Uncertainty of the future**
    - **Fear of infection and health consequences**
- Mainly **depression**, **anxiety disorders**, and **PTSD** were diagnosed significantly more
- Already existing **disparities** were **exacerbated** by the pandemic :
    - **Lower socioeconomic status**
    - **The elderly**
    - **Ethnic minorities**
- These groups were all **affected more** which caused a larger **lacking** of **resources** and **access**
- **Pandemic called for**
    - The need for **investment** in mental health
    - The **inclusion** of long-term mental health **costs** in public policies
- In the US, **60%** of healthcare spending is **associated** with **mental health** disorders
- Even in more **developed healthcare systems** (such as those in western Europe) were required to **increase expenditures** to meet the **increasing demand** for mental health services

Financial Stats & Targets

Atlas
Targets
Economic & Social Factors

1) What is the current average allocation (%) of health funds to mental health care? (Worldwide + NZ)
    a) WHO,(2024/25): The global median government allocation to mental health is roughly 2% of total health budgets (unchanged since 2017). HICs spend up to about US$65 per capita, LICs as low as US$0.04.

    b) New Zealand (2024/25): Minister for Mental Health confirmed ring-fenced funding roughly NZ$2.6 b for 2024/25, which a Parliamentary committee notes is 9% of Vote Health (up from NZ$2.4 b). roughly NZ$490 per person.
2) How much funding is needed to create quality mental health care systems? (Worldwide + NZ)
    a) Benchmarks / targets: Multiple frameworks call for greater than 10% of health spending in HICs and greater than 5% in LMICs devoted to mental health (often used as a near-term "quality system" benchmark).

    b) Global gap (orders of magnitude):

        i) US$200–350 b investment gap to scale evidence-based mental health interventions worldwide (McKinsey Health Institute estimate).

        ii) Development Assistance for Mental Health (DAMH) should rise to ≈US$1.9 b/yr to support LMIC systems (advocacy target used by UnitedGMH).

        iii) ROI evidence: scaling depression/anxiety treatment yields ≈4:1 economic return. This underpins the case for higher, sustained investment.

    c) New Zealand: At 9% of Vote Health, NZ is within striking distance of the 10% HIC benchmark, but performance reviews still flag service gaps (esp. youth and community capacity). Govt spends NZ$2 b+ annually on MH services; 2019's Wellbeing Budget added NZ$1.9 b over five years; recent updates continue baseline uplifts, but delivery and outcomes remain a focus.
3) What would the specific breakdown of funding be? [Eg. Telehealth services, Inpatient Facilities, Staffing, etc.] (Worldwide)
    a) Workforce & community-based service delivery – 45–55%
    Recruit/retain psychiatrists, psychologists, MH nurses, social workers; scale mhGAP task-sharing in primary care; ongoing supervision & CPD.

    b) Primary-care integration incl. psychotropic medicines & talking therapies – 15–20%
    mhGAP-aligned packages, brief psychological interventions, essential meds at PHC level.

    c) Crisis, acute & inpatient capacity (in general hospitals) – 10–15%
    Short-stay acute units, step-up/step-down beds; avoid over-reliance on standalone mental hospitals (Atlas shows many countries are still hospital-centred).

d) Digital/tele-mental health & referral coordination – 5–10%
Telehealth platforms (Atlas: 63% of countries report tele-MH available—scale what works).

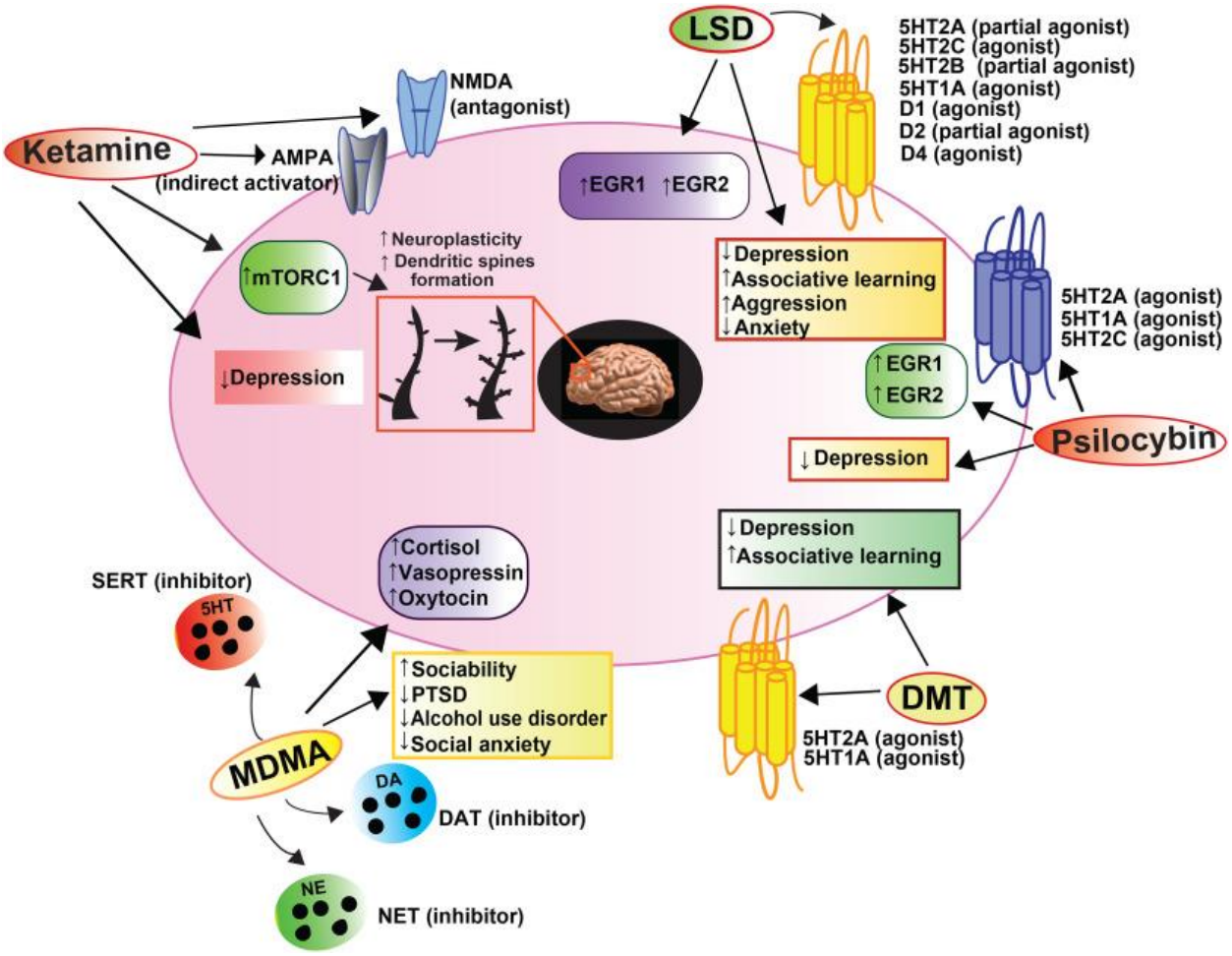
e) Public mental-health & prevention (schools, workplaces, suicide prevention) – 5–10%
Matches Action Plan targets for promotion/prevention programmes.

f) Data systems, quality, M&E, and research – 3–5%
Routine indicators, outcomes tracking, safety/quality improvement; supports ROI-positive scaling.

4) Possible HIC and LIC/MIC partnerships
5) Possible sources for funding (outside of national/international health budgets)

Psychoactive Substances

- Treatment resistant depression – Ketamine Therapy
- Mood regulation - psilocybin and LSD
- PTSD Treatment - MDMA



- Psychedelics can enhance association learning – particularly for disorders that limit it such as MDD (major depressive disorder)
- Psychedelic administration produces a period of behavioral flexibility in which new coping strategies can be learned
- LSD and psilocybin increase interaction between sensory and somatomotor brain networks, and decrease communication among associative brain regions
- May help to reduce rigid thinking often present in mentally ill individuals
- Psychedelics increase visual pathway excitability
- Ketamine - after 4hrs effects may be present
    - 1 week period of efficacy

- Significant abuse liability, and its long-term use is associated with notable bladder and neurologic toxicity

Opening Speech

Honorable Chairs and Fellow Delegates, the delegate of New Zealand strongly affirms the development of accessible, affordable, and quality mental health care with a focus on human and patient rights – especially in the face of crisis. Following the COVID-19 pandemic, the UN developed guidelines that will help all societies gain access to patient-focused and comprehensive mental healthcare. Following these recommendations, New Zealand encourages the decentralization of care especially in areas with fewer healthcare facilities ensuring indigenous populations are not left behind. New Zealand acknowledges the overwhelming burden on psychiatrists, psychologists, and social workers that can be relieved through task sharing and recruitment programmes. New Zealand has begun to model promising infrastructure improvement, through the development of 16 key infrastructure projects, New Zealand aims to alleviate the strain on mental health systems and combine care under a single foundation. Meanwhile, the development of individual facilities with multiple security levels allows for broader oversight while maintaining safety and maximizing efficiency and resources, ensuring care is sustainable and personalized. According to a 2006 study, 20% of New Zealanders with mental health conditions can and do recover – but that relies on a system that's organized, staffed, and funded. New Zealand calls on nations such as; Australia, Canada, Norway, Sweden, and Switzerland to help develop partnerships with less developed countries to provide support through funding, resources, and literacy programmes. Thus, New Zealand urges all nations to consider the reallocation of healthcare funds, consistent reporting on mental health, and breaking down existing cultural biases and stigmas as a part of strengthening mental healthcare to ensure all people receive the care they need and are entitled to. The delegate yields the floor back to the chairs.

Position Paper

New Zealand : WHO (STAMUNXI)

Resolution Ideas

- Systemic Reform
    - Integration of care
        - Stepped care
- Digital care
    - Telepsychiatry
- Acknowledging Severe Workforce Shortages
- Focus on High-Need Populations
    - Youth
- Workforce initiatives
    - Recruitment programs
    - Task sharing
- Public education
    - Public conferences and programs
    - Educational curriculums including mental health literacy
- Tripartite Advisory (as seen in Singapore)
    - Advisory to employers
        - Flexible work arrangement (FWAs)
        - Designating workplace mental well-being champions
        - Providing access to counselling services
- Mass counselling goals
- Screening
    - Young people are entitled to mental health check-ups every 2 years
- Joint response teams
    - 24 hours joint response centers
        - Involving mental health specialists and police officers and establishing emergency dispatch centers for efficient access to emergency care
- Commissioner
    - Appointing a national mental health commissioner
    - Responsible for ensuring mental health is considered in planning and public policies
- Whole-Of-Society Approach
    - Mandates various authorities to implement mental health considerations in policies and varying contexts
- Regional Autonomy
    - Minimized and limited by overarching policies that apply to entire nations or states
- Telehealth Acceleration
    - Accepting telehealth as a valid form of healthcare and covering costs as seen appropriate
- Funding
    - Regulated funding and proportional allocations of health budget between physical and mental health
- Primary Prevention
    - Implementing age and population specific mental health promotion programs
- Integrated Care Systems

    - Local systems responsible for integrating mental and physical health services
    - Ensures parity of esteem (equal priority)
- Social Prescribing
    - Linking depressed and anxious people with various community groups (such as; exercise groups. Gardening, or debt advice) to address special determinants of mental health issues
- Employment Support
    - Individual placement and support (IPS) are committed to helping individuals with mental illness find and keep meaningful employment
- Certified Community Behavioral Health Clinics (CCBHCs)
    - Funding to expand services like CCBHCs
    - Required to provide 24 hour clinic care, screening, and diagnosis
        - Regardless of capacity to pay
- Macro-level stressors
    - Hyperinflation, generalized poverty, food and resource shortages, and political unrest are all factors that contribute to poor public mental health
- Alternative Treatment Methods
    - Psychedelic drugs for specific mental illnesses
- Developing wards or sections in prisons that are specifically for mentally ill inmates that are deemed at risk in normal units
    - Serves as a temporary point of contact until permanent placement can be found or treatments to reintegrate into society can be provided
        - Only in the case that the crime was committed as a result of mental instability or illness
    - Wards will be equipped with multiple mental health professionals with a background in criminology to ensure decisions are made comprehensively and with all aspects in mind
- Military mental health support
- Frontline mental health support

Clause Ideas

POIs

This delegate would like to ask through the chairs,

- How does the delegate plan on ___?
- How does the delegate aim to __?
- Why does the delegate believe __ is necessary?
- What funding does the delegate intend on receiving?
- How does the delegate believe this will have a positive impact?

POCs

This delegate would like to ask through the chairs,

- What does the delegate mean by __?
- Can the delegate elaborate on the impact of __?

- **POI - point of information**
    - **This delegate would like to ask through the chairs ___**
    - Follow up (wait for it to be granted, 1x only)
- **POC - point of clarification**
    - **This delegate would like to ask through the chairs ___**

Topics

- Addressing access

For Speeches

Against Speeches

Main Sub Speeches

Other Speeches

Topics :

- **Breaking down stigmas related to mental health**
- **Ensuring mental health care is accessible and affordable**
    - Ensuring proportionate funding to mental and physical healthcare
- **Combatting the shortage of mental health professionals**
- **Using technology to bridge the access gap for those in rural or less developed areas**
    - Integrating mental health services into communities and general care
- **The development of alternative treatment methods for mental illness**
- **The impact of the COVID-19 pandemic on mental health**
- The implementation of national and international mental health initiatives
- **The consideration of mental health in the justice system**
- **The effectiveness of current inpatient facilities**
- Patient abuse during long-term inpatient stays
- Professional safeguards and training to ensure boundaries between patients and caregivers
- The integration of substance abuse programmes as a part of mental health infrastructure

For Speeches

This resolution stands for all human rights and full inclusivity of all individuals regardless of personal limitations and background. Furthermore, this resolution specifically emphasizes education and employment to ensure all people have equal opportunities to employment and are given accommodations when necessary. This resolution encourages the early implementation of mental health literacy to prevent the development of mental health issues. This resolution strongly encourages regular surveys of mental health status and mental health care to ensure relevant and effective progress is being made. Finally, this resolution considers the various factors impacting mental health, specifically highlighting macro-level stressors like hyperinflation, migration conflict, political unrest, and more. This resolution addresses all individuals, and encourages members of this committee to consider the benefits of this resolution for all nations and populations.

Against Speeches

Other Speeches

Breaking Down Stigmas Related to Mental Health

How can we expect to treat those in need with empathy and care when judgement begins before they walk in the door? The biggest challenge in the face of mental health infrastructure and development is stigma. It prevents progress and creates inequitable access to care. New Zealand strongly believes that by breaking down stigmas and understanding the root causes of misconceptions will serve as a huge step towards a world where mental health care is what it must be: a fundamental human right that is affordable, effective, safe, and accessible. Understanding stigmas and the root causes is the first and most crucial step in breaking down the stereotypes and fears that surround mental health. New Zealand affirms the role of all people in reducing stigmas in mental health systems, through progressive and empathetic legislations – such as; ensuring mental illness does not unnecessarily prohibit participation in society like access to education, voting rights, and equal labour opportunities. Furthermore, New Zealand encourages the development of various community-based and run systems for the support of mentally ill individuals, and their loved ones to help people come to terms with and adapt to their diagnosis or symptoms – through reducing blame and encouraging acceptance and accommodation. Thank you

Ensuring Mental Health Care is Accessible and Affordable

Quality care is nothing if it is not accessible and affordable.

Ensuring Proportionate Funding to Mental and Physical Healthcare

Combatting the Shortage of Mental Health Professionals

A major issue faced when addressing mental health care is the extreme shortages in both resources and professionals. New Zealand encourages nations such as; Norway, Russia, and Switzerland – who have high professional density or per capita ratio – to encourage national mental health specialists to relocate and assist in providing mental health care in less developed countries who will be encouraged to provide visas and other necessary documentation to work legally in the foreign country, especially those lacking an overwhelming amount of specialists and social workers. Furthermore, the shortage of mental health professionals can be rectified through task sharing involving the retraining of community care workers to handle common

mental illnesses and reduce the strain on psychiatrists and psychologists ensuring severe cases are able to get the attention they require. Lastly, engaging in early intervention strategies and prevention strategies will reduce the demand for mental health services – specifically for severe and complex cases. New Zealand urges all nations to collaborate and share resources and strategies to solve this crisis of inadequate numbers of mental health professionals. Thank you

Integrating Mental Health Services into Communities and General Care

Using Technology to Bridge the Access Gap for Those in Rural or Less Developed Areas

Healthcare access continues to be an issue all over the world whether due to a shortage of resources, professionals, or poorly distributed care. New Zealand strongly encourages the development of partnerships between HICs and LMICs to exchange resources and funding as well as strategies for early intervention and prevention initiatives. Through donations and fundraising, HICs can help provide necessary technology to LMICs – especially to less connected areas and rural regions. New Zealand encourages the creation of internationally regulated digital platforms for access to mental health care and connection of patients with hospitals or care facilities – reducing the need for consultations or in person visits. Furthermore, the use of technology in healthcare can be used to provide care for highly anxious individuals that may be prevented from accessing in person care due to the overwhelming environment of the outside world.

The Development of Alternative Treatment Methods for Mental Illness

The Impact of COVID-19 on Global Mental Health

The Implementation of National and International Mental Health Initiatives

The Consideration of Mental Health in the Justice System

Patient Abuse During Long-Term Inpatient Stays

Professional Safeguards and Training to Ensure Ethical Standards

Mental Health in Youth and Literacy Initiatives

Questions

1. **What are you looking for in a best delegate?**
2. **What do I need to keep in mind?**